

Informational Note

These criteria include the following procedures:

Celiac Plexus Neurolysis for

- Pancreatic cancer with abdominal or back pain

- Chronic pancreatitis with chronic abdominal or back pain

Chemical Neurolysis for

- Severe cancer pain

Cooled Radiofrequency Ablation for

- Sacroiliac (SI) joint pain

Electrical Neurolysis for

- Trigeminal neuralgia

Facet Neurotomy for

- Cervical facet joint pain

- Lumbar facet joint pain

- Thoracic facet joint pain

Facet Rhizotomy for

- Cervical facet joint pain

- Lumbar facet joint pain

- Thoracic facet joint pain

Nonpulsed Radiofrequency Ablation for

- Cervical facet joint pain

Radiofrequency Neurolysis or Neurotomy for

- Cervical facet joint pain

- Lumbar facet joint pain

- Sacroiliac (SI) joint pain

- Thoracic facet joint pain

- Vertebrogenic low back pain

Superior Hypogastric Plexus Neurolysis for

- Severe cancer pain

Therapeutic Medial Branch Radiofrequency Neurotomy for

- Cervical facet joint pain

- Lumbar facet joint pain

- Thoracic facet joint pain

Thermal Destruction of Intraosseous Basivertebral Nerve for

- Vertebrogenic low back pain

These criteria cover indications for destruction of nerves by a neurolytic agent (e.g., chemical, electrical, radiofrequency, thermal). Injection of an anesthetic agent for diagnostic or therapeutic purposes (sympathetic blockade) and surgical transection of the sympathetic nerves (sympathectomy) are not covered by these criteria. For nerve block or surgical procedures performed on the sympathetic chain, see the "Sympathetic Blockade" or "Sympathectomy" criteria subset. For facet joint injections, see the

"Facet Joint Injection" criteria subset. For sacroiliac joint injections or sacroiliac nerve block, see the "Sacroiliac (SI) Joint Injection" criteria subset.

While there is some evidence for the use of pulsed radiofrequency ablation (RFA) for intercostal neuralgia and post thoracotomy syndrome, evidence is lacking for use of pulsed RFA in the spine and sacroiliac joint and is, therefore, not covered in this criteria subset.

Percutaneous neuroablation is a procedure that destroys or damages neural tissue to relieve pain. These procedures may be performed using one of several mechanisms. Chemical neuroablation involves the percutaneous administration of phenol or alcohol around a nerve to denervate it. Radiofrequency neuroablation is a type of thermal neuroablation utilizing heat. Cold neuroablation (e.g., cryoneurolysis, cryoablation) is experimental and currently not addressed by these criteria.

Imaging guidance is necessary for safe and successful needle placement. Fluoroscopy is most commonly used, although CT has also been used to guide needle placement. The use of ultrasound to perform these procedures is considered controversial and is, therefore, not covered by these criteria. ([1](#), [2](#), [3](#), [4](#))

InterQual® Procedures criteria are derived from the systematic, continuous review and critical appraisal of the most current evidence-based literature and include input from our independent panel of clinical experts. To generate the most appropriate recommendations, a comprehensive literature review of the clinical evidence was conducted. Sources searched included PubMed, Agency for Healthcare Research and Quality (AHRQ) Comparative Effectiveness Reviews, the Cochrane Library, Centers for Medicare & Medicaid Services (CMS) National Coverage Determinations, and the National Institute for Health and Care Excellence (NICE). Other medical literature databases, medical content providers, data sources, regulatory body websites, and specialty society resources may also have been used. Relevant studies were assessed for risk of bias following principles described in the Cochrane Handbook. The resulting evidence was assessed for consistency, directness, precision, effect size, and publication bias. Observational trials were also evaluated for the presence of a dose-response gradient and the likely effect of plausible confounders.

[1. Hurley et al., Reg Anesth Pain Med 2022, 47: 3-59](#)

[2. Cohen et al., Reg Anesth Pain Med 2020, 45: 424-67](#)

[3. North American Spine Society \(NASS\). Evidence-Based Clinical Guidelines for Multidisciplinary Spine Care: Diagnosis and Treatment of Low Back Pain. 2020](#)

[4. North American Spine Society \(NASS\). NASS Coverage Policy Recommendations: Facet Joint Interventions. 2016](#)